

especially if it increases numbness; consider resting your forearms on the floor or blocks, or try using a wedge.

DEGENERATIVE DISK DISEASE

Practice spinal flexion and spinal rotation gently; avoid or be careful during headstands, shoulderstands, or anything that puts pressure on the neck.

DIABETES

For type 1, avoid anything that puts pressure on your insulin pump; for type 1 and 2, eat before class if needed, and rest if lightheaded.

DISK HERNIATION (SLIPPED, BULGING, PROTRUDING)

Be careful of unsupported spinal flexion, such as a Standing or Seated Forward Fold, and spinal rotation; focus on lengthening the spine before gently entering a pose, and consider keeping the spine neutral and bending at the hips into a Forward Fold—Child's or Cat pose may be safer forms of spinal flexion; be careful during headstands, shoulderstands, or anything that puts pressure on the neck.

EAR INFECTION

Be careful with inversions and in balancing poses.

EYE CONDITIONS THAT

INCREASE PRESSURE (such as glaucoma, detached retina, diabetic retinopathy, or recent cataract surgery)

Be careful with or avoid any pose in which the head goes below the heart, holding the breath (kumbhaka), and fast breathing (kapalabhati); seek the advice of your ophthalmologist if you are unsure.

FIBROMYALGIA

Consider restorative yoga and yoga nidra; use lots of props and let your teacher know if you prefer not to be touched in a hands-on assist.

FROZEN SHOULDER (ADHESIVE CAPSULITIS)

Move slowly into shoulder stretches and gradually increase the stretch over time.

HEART CONDITIONS

Be careful when performing inversions, holding the breath (kumbhaka), and fast breathing (kapalabhati); you should also seek the advice of your cardiologist.

HIGH BLOOD PRESSURE (HYPERTENSION)

Be careful with any pose where the head goes below the heart, holding

the breath (kumbhaka), and fast breathing (kapalabhati); if your blood pressure is not currently regulated, avoid full inversions, intense practice, and hot yoga completely.

HIP REPLACEMENT

Follow these precautions 6–8 weeks after surgery, and with the advice of your doctor. In the anterior approach, be careful of or avoid extension (as in the lifted leg in Warrior III); in the posterior approach, be careful of or avoid hip flexion past 90 degrees, internal rotation, and crossing the midline (crossing legs); after proper healing, you are likely to be able to perform all of these movements, but move slowly into the poses and ask your doctor for advice.

HYPERMOBILITY

Avoid any extreme movement or hyperextension of joints; focus on strengthening.

KNEE LIGAMENT INJURY (ACL, PCL, LCL, MCL)

Be careful with poses that involve rotation (e.g. Triangle pose and Warrior II); for ACL, avoid deep knee flexion and for PCL, be careful of hyperextension/locking your knees; for both, be careful of or avoid jumping into poses.

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